ENSIGN SERVICES

POSTette: Establishing and Modifying Goals

Reference: Rehabilitation Services P&P- Policy 235 –The Progress Report; Medicare Benefit Manuals Chapters 8 & 15

Establishing Goals
Documentation must demonstrate progress toward achievement of goals identified on initial evaluations, and a patient's response to intervention. The reasonableness of time, frequency and duration of intervention must be documented in order to be reimbursed for care.

Goals need to be patient driven, directly tied to functional limitations, related to a functional activity that can be measured, & have a specified time frame.

When establishing patient goals, it is helpful to tie back to the deficits identified on the evaluation and results of standardized testing.

Medical Necessity
What is the reason for intervention?
Establish goals to address the issue. What steps will be used to improve language/ mobility/ ADLs?

Reasonable and Necessary

Reasonable – the objective relates in a realistic manner to the potential the patient possesses, and are achievable in a period of time that corresponds to the severity of impairment and the potential for recovery.

Necessary- there is a reason the patient needs to accomplish the goal.

- Elicit positive medical/functional improvement (function must be tied to the expectation to perform the activity – people living in SNF's don't need to manage money, people living with 24-hr caregivers have available supervision, etc.)
- Prevent adverse medical impact/increased dependence related to the documented impairment

Skilled Intervention
The interventions provided must tie back to the established goals. Do the established goals match the planned intervention?

Building a Goal

WHO: The patient is always the recipient of our services. Goals should never be set for "caregiver to utilize …" or "therapist to determine …."

WHAT: What the patient will accomplish

HOW: Objective criteria used to measure goal

IN ORDER TO: Final outcome expected

HOW LONG: Anticipated time it will take to achieve established goal. (i.e. 2- 4 weeks… STG; 8-12 weeks LTG)

Long Term Goal (LTG)
When establishing the LTG determine what was the patient's PLOF as it relates to each discipline and is the clinical plan to return to that level? If yes – that is your long term goal

NOTE: Goals typically would not exceed PLOF unless the clinical case of the patient is clearly captured in the documentation to justify why skilled intervention is targeting that outcome. UPOCs will be needed to justify on-going care.

If No – your long term goal will focus on the anticipated discharge level.

- Describes the functional level that the therapist expects the patient to achieve at the time of discharge from the particular care setting.
- Are designed to cover the entire course of treatment OR a Certification Period (preferable entire course of treatment)
- **NOTE:** A change in a LTG requires an updated plan of care.

Short Term Goal (STG)
When establishing the STG(s) determine what steps need to be attained to meet the LTG. They should be established for an interval (defined as 1 -4 weeks and typically not to exceed 30 days)

- Relatively small, interim achievements that serve as measurable milestones, as the patient works his/her way toward long-term goals.
- They will be modified throughout the course of treatment
- The goals will determine what daily interventions will occur
- Goals need to be individualized and customized to each patient.
- Avoid using too many builds.
- Focus on the most important tasks for each progress report period. As intervention occurs and additional data is collected, the goals can be easily modified and updated when completing progress report.

Progress Reporting
Medicare will only cover services if it is clear that a therapist must provide the care that allows the patient to make progress. This means that the patient wouldn't recover spontaneously if given rest and time, AND that other less skilled personnel couldn't achieve the same outcome.

Progress reports need to show improvement or support skilled services provided.

- Include objective data related to the goals
- Provide an assessment of improvement or progress toward goals.
    - Changes in the level of assistance required to perform functional tasks
    - Changes in the types of functional activities/ tasks
    - Changes in the types of assistive devices
    - Improvement in rating of reported pain levels and changes in the ability to perform tasks given the reduction of pain (i.e. Ability to sit

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for a duration of time as a result of pain reduction)

- Make considerations in the documentation for medically complex, cognitively impaired or patients not showing significant progress at the time the report is completed.

- Include changes or modifications to goals
- If limited or no progress, use comments to further explain / describe why services will continue.

NOTE: Report outcomes using the same data as in goals. If percentage used in goal – use percentage when reporting data.

As a rule of thumb: If there isn't any identifiable progress after 2 weeks then continuation of service without evidence of need for skilled intervention will likely be denied.

NOTE: Net Health allows tracking of the amount of cuing as progress, but it should be noted that this alone is not considered "material impact," unless the reduction is a significant percentage or alters the amount of physical support needed by the patient (close tactile to supervision, for instance.)

Pt will perform sit to stand transfers from her power chair with SBA 5 reps in 1 min, to increase her independence in transfers with care staff.

Baseline (7/16/2015)	Previous (7/29/2015)	Current (8/4/2015)
Currently pt using sit to stand machine with care staff.	Pt is able to perform 2 STS in 1', from raised bed pulling up from 4WW.	5 reps in 2 min

STG #1.0 - Goal Met

Pt will come to stand from sitting with modA in preparation for ADLS.

Baseline (7/22/2015)	Previous (8/11/2015)	Current (8/18/2015)
MaxA	Pt able to sit-stand CGA. Continue x one more week to ensure carryover - pt was maxA last reporting period.	Pt able to stand CGA with vc's

STG #1.0 - Goal Met

Patient will increase use of breath support and control strategies to to 90% accuracy during production of 10 functional words increase speech intelligibility.

	Baseline (8/3/2015)	Previous (8/3/2015)	Current (8/10/2015)
Percent	75%	75%	90%

Comments: demonstrates ability to verbalize safety however presents with decreased accuracy with performance

TIP: Use comments section for goals to further support any justification for continuation especially when progress is not clearly demonstrated from previous progress report.

Considerations for patients who are clinically complex:

Determine how all of the information collected can be captured in functional goals. Goals need to be measurable, functional, and sustainable. Goals for this population need to address:

- Improving the patient's ability to perform activities of daily living
- Decreasing symptoms identified in evaluation that impact function.
- Increasing endurance and strength
- Improving the patient's quality of life
- Decreasing negative consequences of deconditioning
- Returning patient to prior level of function (or beyond)
- Include the patient and family to determine functional goals for discharge
- Implement small incremental goals that will be updated frequently for this population. (Modification of the goals and treatment plan are skilled services).

Goal examples:

- In 2 weeks, patient will patient will utilize necessary adaptive equipment to safely complete upper body dressing tasks increasing to minimal assistance while maintaining oxygen saturation levels > 90% with minimal verbal cues for breathing techniques.
- In four weeks patient will demonstrate increased activity tolerance as demonstrated by ability to have a 15 minute phone conversation with O2 saturation > 90%

Considerations with cognitively impaired patients: Goals may be focused on improving safety during functional tasks and structuring care to allow the patient to perform at their best functional ability consistently during activities.

Goal examples:

- To improve ambulation safety the patient will demonstrate the ability to anterior weight shift with a 3' functional reach without evidence of fear or instability.
- Patient will complete tooth brushing task with 25% verbal cues for sequencing and adapted task set up with the use of striking visual cues

Clinical Example: Patient with pre-existing mild-moderate cognitive impairment in a long term setting, fully disoriented and unable to participate in previously enjoyed activities after medically complex hospitalization.

Potential Goals:

- Patient will be 100% oriented to three key routes within the facility to independently access meals, activities and nursing staff.
- Patient will be able to follow simple 3 part instructions with 100% accuracy during dressing and meal set-up tasks to decrease risk for progressive physical dependence on caregivers during daily activity.

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- Patient will be able to use bedside informational sheet to orient to location and staff when cued with 100% accuracy to decrease confusion and anxiety.
- Patient will sustain attention to adapted activities for 10 minutes after set-up by staff to facilitate participation with recreational tasks in order to maintain cognitive abilities and prevent isolation.

Tip 1: It is important that the duration of treatment needed to reach these goals correlates appropriately to amount of change (both originally projected and then actually provided).

Tip 2: Any goal that will ultimately require heavy caregiver assist should include the participation of the caregivers almost immediately.

Considerations for patients with Dementia:

- Encouraging the continuation of lifelong roles
- Encouraging socialization
- Improving the level of alertness
- Assisting with reality reassurance
- Maintaining motor skills and/or stimulate motor responses
- Maintain verbal skills and/or stimulate verbal responses
- Providing an outlet for creativity and self-expression
- Encouraging self-directed activity to replace lost skills
- Promote surrounding awareness

Low-Level/Foundation Goals

Patients with marked impairments that worsen after an illness or hospitalization will often still benefit from treatment, so it's important to be able to create goals that are reasonable and necessary to capture the material impact the expected treatment to have.

Problem: Patient was non-ambulatory, max assist for transfers and TD for ADL's before, but is now fully bedbound.

Potential Goals:

- Improve LE strength from 2-/5 to 3-/5 in order to perform Max A transfers to/from bed without requiring a mechanical lift.
- Dynamic sitting balance from TD to Poor+ to improve trunk stabilization during transfers and decrease risk for personal and/or caregiver injury.
- Supporting sitting from unable to 5 minutes to allow sponge bathing of back.
- Increase out of bed tolerance from unable to 2 hours via structured up/down schedule and progressive decrease in postural support of wheelchair.

Using Test Measures in Goals

Remember that a test measurement is not a functional ability, and that while test scores *correlate* to function they cannot take the place of a description of function.

Test scores are effective at objectively demonstrating change, but should be used as a data point to show the change rather than using the improvement in the test score as goal itself.

Incomplete goal: Patient will increase temporal orientation to 27/30 on the RIPA-G subtest. (In order to do what? Why is this significant?)
Complete goal: Patient will be orientated to place and time 8/10 trials when verbally prompted to use an environmental cuing system.

This can then be supported by reporting a change in the subtest score at the recertification.

Incomplete goal: Patient will increase functional abilities as demonstrated by an increase on the Barthel Index to 35/100. (What functional abilities? To do what?).

Complete goal: Patient will complete Upper Body ADL's at overall supervised levels to allow safe return to home setting with family.

Remember to include:

- Modification of short term goals or treatment plan (remember changes to LTG requires an UPOC)
- Skilled services provided and improvements in functional skills using comparative data or additional documentation for patients with limited progress.
- Any current deficits or barriers and include how the barriers will still support the need for skilled intervention.
- Justification for continuation even if progress is clearly identified with the goals data

Examples of denial of services due to services not having supportive goals:

- OT documenting transfer training and bed mobility, however, 1) PT was already working on those clinical areas, and 2) there were no goals to support those areas.
- PT writing goals for trunk control and transfers. Documentation was for gait training (without a goal), and minutes logged for W/C management (without any goals).
- ST completing a dysphagia eval and identifying the dysphagia treatment dx. However, writing goals for and treating speech intelligibility (including MIT for apraxia). No motor speech assessment, no dysarthria/apraxia treatment dx, and no ST treatment code (logged all minutes under 92526).

Additional Support

Medicare Benefit Manual Chapter 15
POC & UPOC POSTettes

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